

Policy on Prevention of Healthcare Associated Infections in Staff

«Hospital Name»

Document control

Document No.	HCO/IPC/POL/08	Version	2.0
Issue No.	01	Review due	One year from implementation
Date created	_____	Date of implementation	_____
Prepared by	Infection Prevention and Control Officer Name: _____ Signature: _____		
Reviewed by	Quality Coordinator Name: _____ Signature: _____		
Approved by	Medical Superintendent Name: _____ Signature: _____		

A blank marked _____ must be completed before issue.

Statement of intent

«Hospital Name» implements occupational health and safety practices as per written guidance, provides Hepatitis B vaccination to all direct-patient-care staff and other relevant immunisation as per risk, implements work restrictions for staff with transmissible infections, implements blood and body fluid exposure prevention measures, and provides post-exposure prophylaxis for Hepatitis B and HIV exposure the same shift.

1. Purpose

This policy explains how «Hospital Name» takes action to prevent or reduce healthcare associated infections in staff: implementing occupational health and safety practices as per written guidance; maintaining a staff immunisation programme with a Hepatitis B vaccination floor; implementing work restrictions for healthcare providers with transmissible infections; preventing blood and body fluid exposures; and providing post-exposure prophylaxis aligned with national/international guidelines.

Standard precautions, hand-hygiene guidelines, and IPC resources are covered in the hospital's other policies.

2. Scope

This policy applies to all staff — clinical, nursing, allied health, laboratory, housekeeping, CSSD, and support staff — working at «Hospital Name», and to the Occupational Health Physician and the IPC Officer in their roles under this policy.

3. Policy standards

«Hospital Name» maintains written guidance on occupational health and safety practices and provides adequate resources to staff for IPC. Pre-employment health screening and capture of immunisation status are conducted where appropriate. The immunisation policy is in consonance with available evidence; at minimum, Hepatitis B vaccination is provided to all staff in direct patient care; other relevant immunisation is provided as per risk and statutory requirements. Work restrictions for healthcare providers with transmissible infections are implemented; the organisation encourages reporting of illness and does not penalise staff for reporting. Healthcare providers use appropriate PPE to prevent blood and body fluid exposures; the organisation strives to eliminate needle devices when safe alternatives exist; sharps containers are at the point of use and replaced before full. PEP for Hepatitis B and HIV exposure is provided the same shift, aligned with national/international guidelines, and documented by the IPC Nurse.

Staff follow the written guidance below and keep the records it requires.

4. Non-negotiable rules

- Do not continue clinical duty when a work restriction for a transmissible infection applies to you.
- Do not leave a blood or body-fluid exposure — including a needle-stick — without starting the organisation's post-exposure path the same shift.
- Do not operate without Hepatitis B vaccination being available to and provided for all staff in direct patient care.
- Do not leave a staff member's exposure event undocumented in the IPC Nurse's PEP record file.
- Do not penalise any healthcare provider for reporting illness, an exposure, or observing a colleague's work restriction.
- Do not recap used needles — place sharps in the puncture-proof container at the point of use immediately after use.

5. What we do

5.1 Implement occupational health and safety practices as per written guidance

Written guidance on occupational health and safety practices is available and implemented at «Hospital Name». The guidance covers PPE use, access to vaccination, exposure

reporting, and work restriction for transmissible infections. The organisation provides adequate resources to staff for IPC. Staff adhere to standard precautions as required by the hospital's IPC policies. Where appropriate, pre-employment health screening is conducted and immunisation status is captured. The Occupational Health Physician and the IPC Officer own this guidance jointly. Staff are trained at induction and at least annually.

5.2 Implement a staff immunisation programme

«Hospital Name»'s immunisation policy for staff is in consonance with available evidence. At minimum, Hepatitis B vaccination is provided to all staff involved in direct patient care. Other relevant immunisation is provided as per the risk from time to time and in accordance with applicable statutory requirements. The Occupational Health Physician maintains an immunisation register recording staff name, vaccine, dose, date, and booster schedule. An unvaccinated direct-care worker without a documented contraindication or documented refusal is a gap at this element.

5.3 Implement work restrictions for healthcare providers with transmissible infections

«Hospital Name» implements work restrictions for healthcare providers with transmissible infections. Restrictions limit the role and responsibilities of the affected staff member until they are cleared. Examples of transmissible infections that may trigger restrictions include acute conjunctivitis, chickenpox, acute respiratory infections, and smear-positive tuberculosis; the list is not exhaustive. The organisation actively encourages healthcare providers to report their illness or exposures and does not penalise them for doing so. Evidence of restrictions that were actually applied — not only a policy document — is maintained.

5.4 Implement blood and body fluid exposure prevention measures

Healthcare providers use appropriate PPE to prevent blood and body fluid exposures. «Hospital Name» strives to eliminate the use of needle devices whenever safe and effective alternatives are available. Sharps containers are at the point of use in all clinical areas and are replaced before they are full. A recapping culture on a unit is treated as an incident and investigated. The IPC Nurse includes sharps safety in regular IPC rounds.

5.5 Provide post-exposure prophylaxis

«Hospital Name» provides post-exposure prophylaxis (PEP) to all concerned staff members following blood or body-fluid exposures, including needle-stick injuries. PEP for Hepatitis B and HIV exposure is provided and aligned with national/international guidelines. Immediate first aid at the exposure site is the first step. The Occupational Health Physician or the emergency doctor starts the PEP path the same shift. The IPC Nurse maintains the PEP record file in confidence, with the staff member's name, date, type of exposure, PEP given, and follow-up schedule. Confidentiality follows the hospital's staff confidentiality policy.

6. Stop-work authority

Do not continue clinical duty when a work restriction for a transmissible infection applies to you.

Do not leave a blood or body-fluid exposure (including a needle-stick) without starting the organisation's post-exposure path the same shift.

Stop-work applies to the restricted duty and to delay of PEP. Immediate first aid at the exposure site starts at once.

The person who stops (or the colleague who sees the restriction being ignored) tells the Occupational Health Physician and the Infection Prevention and Control Officer the same shift. Reporting an exposure or observing a restriction is not a disciplinary matter.

7. Governance and responsibility

Role	Responsibility
Medical Superintendent	Accountable for ensuring staff occupational health and IPC protections are resourced and implemented.
Infection Prevention and Control Officer	Owns implementation of this policy; monitors compliance; receives stop-work escalations the same shift; brings staff infection data to the IPC committee.
Occupational Health Physician	Owns the staff immunisation programme and immunisation register; manages work restrictions; starts or coordinates the PEP path on the same shift as an exposure; holds exposure event records.
Infection Prevention and Control Nurse(s)	Maintains PEP documentation file; includes sharps safety and PPE compliance in regular IPC rounds; raises stop-work when a trigger is observed.
Nursing Superintendent	Ensures nursing staff comply with blood/body-fluid exposure prevention measures and report exposures without delay.
Department Heads	Ensure staff in their departments comply with work restrictions when applied and support reporting without fear of penalty.
Quality Coordinator	Audits this policy; holds training records, staff acknowledgements, and stop-work event logs.

8. Quality monitoring

Monitoring area	What is monitored
-----------------	-------------------

Occupational health guidance	Written guidance available and current; pre-employment screening conducted where appropriate; staff trained at induction and annually.
Immunisation programme	Immunisation register current; Hepatitis B coverage confirmed for all direct-patient-care staff; statutory requirements met.
Work restrictions	Records of work restrictions applied; reporting culture evidenced (staff report illness without penalty).
Blood/body-fluid exposure prevention	Sharps containers at the point of use in all clinical areas; regular IPC rounds confirm no recapping; PPE availability confirmed.
PEP provision	PEP record file maintained by IPC Nurse; PEP aligned with national/international guidelines; no missed same-shift starts.
Stop-work events	Stop-work events logged with trigger, action taken, and outcome.

9. Training and staff acknowledgement

All clinical and nursing staff shall be familiar with the occupational health and safety guidance, the immunisation programme, the work restriction process, blood and body fluid exposure prevention (including safe-sharps practice), and the PEP path and stop-work authority in this policy.

I have read the Policy on Prevention of Healthcare Associated Infections in Staff of «Hospital Name». I will follow the processes described, including the stop-work authority in Section 6.

Staff name	Designation	Signature	Date
_____	_____	_____	_____
_____	_____	_____	_____
_____	_____	_____	_____

10. Distribution

This policy shall be available to the IPC Officer, the Occupational Health Physician, IPC Nurse(s), all clinical and nursing staff, department heads, and the Quality Coordinator.

11. Abbreviations

Abbreviation	Meaning
CAPA	Corrective and Preventive Action
CSSD	Central Sterile Services Department
HIV	Human Immunodeficiency Virus
IPC	Infection Prevention and Control
IPCO	Infection Prevention and Control Officer

IPCN	Infection Prevention and Control Nurse
NABH	National Accreditation Board for Hospitals and Healthcare Providers
PEP	Post-Exposure Prophylaxis
PPE	Personal Protective Equipment
TB	Tuberculosis
WHO	World Health Organization

12. Traceability table

This table is an index. It is not how the policy is organised. An asterisk in the Level column means documentation of the process is required.

Objective Element	Level	Traceability to this policy
IPC.8.a	Commitment*	Sections 3 and 5.1 address written occupational health and safety guidance, adequate IPC resources for staff, standard-precaution adherence, and where-appropriate pre-employment screening and immunisation-status capture.
IPC.8.b	Commitment*	Sections 3 and 5.2 address the staff immunisation policy in consonance with evidence, the Hepatitis B vaccination floor for direct-patient-care staff, and other relevant immunisation as per risk and statutory requirements.
IPC.8.c	Achievement	Sections 3, 5.3 and 6 address work restrictions for healthcare providers with transmissible infections, the culture of reporting without penalty, and the stop-work trigger when a restriction is disregarded.
IPC.8.d	Commitment	Sections 3 and 5.4 address PPE for blood/body-fluid exposure prevention, striving to eliminate needle devices, sharps containers at the point of use, and a no-recapping rule.

IPC.8.e	Commitment*	Sections 3, 5.5 and 6 address PEP for Hepatitis B and HIV exposure, alignment with national/international guidelines, same-shift start, IPC Nurse documentation, and the stop-work trigger for delayed PEP.
---------	-------------	---

13. Required Records/Evidence Checklist

Occupational health and safety guidance — IPC.8.a (Commitment*)

- Written occupational health and safety guidance covering PPE, vaccination access, exposure reporting, and work restriction.
- Pre-employment health screening records (where conducted) and immunisation status capture at induction.
- Staff training records for occupational health and safety at induction and annually.

Staff immunisation programme — IPC.8.b (Commitment*)

- Written staff immunisation policy in consonance with available evidence.
- Immunisation register: staff name, vaccine, dose(s), date(s), booster schedule — current for all staff.
- Hepatitis B vaccination coverage record confirming all direct-patient-care staff are vaccinated (or have a documented contraindication or refusal).

Work restrictions — IPC.8.c (Achievement)

- Written work restriction policy/guideline listing conditions and restriction types.
- Records of actual work restrictions applied (case records) — policy alone does not satisfy this Achievement OE.
- Evidence of reporting culture: documentation that staff are encouraged to report illness/exposure and are not penalised.

Blood and body fluid exposure prevention — IPC.8.d

- Sharps container availability records confirming containers are at the point of use in all clinical areas and replaced before full.
- Regular IPC round records confirming no recapping practice observed.
- Needle-stick / exposure incident records and investigation outcomes.

PEP provision — IPC.8.e (Commitment*)

- Written PEP protocol for Hepatitis B and HIV exposure, aligned with named national/international guidelines.

- PEP record file maintained by the IPC Nurse: staff name, date, exposure type, PEP administered, follow-up schedule.
- Evidence of same-shift start for each PEP event.
- Stop-work event log for any delayed-PEP events.

14. References

1. National Accreditation Board for Hospitals and Healthcare Providers. NABH Accreditation Standards for Hospitals, 6th Edition. IPC.8.

Disclaimer

This document is a template prepared for the guidance of «Hospital Name» and must be reviewed, adapted and formally approved by «Hospital Name» before use. Every entry marked [Hospital to define] must be replaced with the hospital's own decision; a document issued with those markers left in place is not an approved policy.

The requirements in this document are accreditation requirements of the NABH Full Accreditation Standards for Hospitals, 6th Edition, not duties under a named Act of Parliament. This policy does not import the Consumer Protection Act, 2019, the Clinical Establishments Act, 2010, or the Mental Healthcare Act, 2017 as a checklist. Statutory duties that arise under other documents of «Hospital Name» remain those documents. «Hospital Name» is responsible for verifying any statutory duty that applies to it; this document does not constitute legal advice.

The clinical and technical content reflects recognised national and international guidance current at the date of preparation. «Hospital Name» remains responsible for verifying that it is current and consistent with the edition of the accreditation standard against which it is being assessed.

This document is not issued by, endorsed by, or affiliated with NABH, the World Health Organization, the National Centre for Disease Control, the Food Safety and Standards Authority of India, any Pollution Control Board, or any other body named in it. Wording is original; no text has been reproduced from the standards, rules or guidelines referenced.